PM-03 Pediatric Hypotension / Shock

CLAIBORNE COUNTY EMS | MEDICAL-DIRECTOR APPROVED | Reconciled October 26, 2026

Recognize pediatric shock before hypotension develops. Use perfusion and clinical trends, not one blood-pressure value or a shock-index shortcut. Treat the likely cause while supporting airway, oxygenation, and circulation.

Recognition and field flow

Step	Approved action
1. Recognize shock	Assess mental status, central/peripheral pulses, capillary refill, skin temperature/color, work of breathing, BP trend, history, and likely cause. Hypotension is a late, ominous sign.
2. Immediate care	Support airway/ventilation, control external hemorrhage, keep warm, check glucose, monitor ECG/SpO2, and obtain rapid IV/IO. Do not delay critical treatment for repeated IV attempts.
3. Identify type	Consider hypovolemic/hemorrhagic, distributive/septic, cardiogenic, or obstructive shock. Use age-adjusted vital signs, perfusion findings, and clinical trends.
4. Fluids	Hypovolemic/distributive: isotonic crystalloid 10-20 mL/kg IV/IO aliquots with reassessment after every bolus. Cardiogenic: 5-10 mL/kg cautiously with early Medical Control.
5. Refractory/obstructive	For fluid-refractory septic shock, contact Medical Control for epinephrine or norepinephrine infusion. Decompress tension pneumothorax and treat other obstructive causes when indicated.
6. Transport	Reassess perfusion, lungs, mental status, glucose, and BP after every intervention. Notify a pediatric-capable destination early and transport without delay.

Age-defined hypotension and provider actions

Topic	Approved action
Hypotension	Neonate: SBP below 60. Age 1 month to younger than 1 year: below 70. Age 1-9 years: below 70 + (2 x age in years). Age 10 years or older: below 90 mmHg.
EMT	Support airway/ventilation, control external hemorrhage, keep warm, assess glucose, apply monitoring when available, recognize compensated shock, and expedite transport.
AEMT	All EMT care plus rapid IV/IO access and shock-type-specific isotonic-crystalloid aliquots with reassessment after every dose. Use IO promptly when access is not quickly available in critical shock.
Paramedic	All prior care plus cardiac monitoring, needle decompression for tension pneumothorax, and Medical-Control-directed vasoactive infusion for fluid-refractory septic shock.

Immediate safeguards

• Reassess after every fluid bolus for perfusion, lungs, hepatomegaly, work of breathing, and other signs of volume overload.

• Avoid routine 20-mL/kg boluses in suspected cardiogenic shock. Open PC-03 Pediatric Pulmonary Edema / CHF and use small, cautious aliquots.

• Treat the cause: control hemorrhage, give epinephrine for anaphylaxis, decompress tension pneumothorax, and use the appropriate cardiac or airway pathway.

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FLUIDS, ADRENAL-CRISIS SAFEGUARDS, AND HANDOFF

Fluid, vasoactive, and emergency steroid pathway

Therapy	Dose and role
Hypovolemic/distributive	Isotonic crystalloid 10-20 mL/kg IV/IO aliquots. Reassess after every bolus; further fluid is individualized with Medical Control.
Cardiogenic	Isotonic crystalloid 5-10 mL/kg IV/IO cautiously with early Medical Control. Reassess after each aliquot and monitor for pulmonary edema.
Vasoactive infusion	Epinephrine or norepinephrine infusion is Medical-Control-directed for fluid-refractory septic shock. Verify pediatric formulation and dose in the current medication reference.
Adrenal crisis	For known or suspected adrenal crisis, use the patient's prescribed emergency steroid plan when available. Otherwise use a Medical-Control-directed equivalent. Do not use routine corticosteroids for septic shock.

Cause-specific pathway and reassessment

• Hypovolemic/hemorrhagic: control external bleeding, package for rapid transport, and use the applicable trauma/hemorrhage pathway.

• Distributive: use PM-01 for anaphylactic shock; assess for sepsis, neurogenic causes, medication/toxin effect, and adrenal crisis when history or emergency plan supports it.

• Obstructive: assess for tension pneumothorax, tamponade, and other obstructive causes. Treat immediately when an approved intervention is indicated.

• Document serial mental status, vital signs, perfusion, fluid volume, lung findings, vasoactive/airway interventions, consultation, and response after every intervention.

Transport, documentation, and linked pathways

Topic	Required action
Transport	All children with suspected shock require prompt transport. Escalate destination notification for persistent poor perfusion, airway compromise, cardiogenic shock, suspected sepsis, or vasoactive support.
Handoff	Communicate likely shock type, precipitating history, age/weight source, serial perfusion and vital signs, fluids/medications with response, airway support, consultation, and destination notification.
Links	AR-05 Pediatric Airway; PC-03 Pediatric Pulmonary Edema / CHF; PM-01 Pediatric Anaphylaxis; current Claiborne Medication Reference.

Critical reminder: Hypotension is late in pediatric shock. Reassess after every intervention, avoid excess fluid in cardiogenic shock, and contact Medical Control early for fluid-refractory shock or an adrenal-crisis treatment decision.

Clinical pearls

• Early pediatric shock is recognized by perfusion and behavior changes before hypotension develops.